Sudden Shock

Psychiatric injury in negligence in England and Wales. The historical shock requirement, Sion and Ronayne, and the correction made in Paul [2024] UKSC 1.

1. Read the old language in its historical setting

Earlier cases often described a secondary victim's illness as the result of **nervous shock**: an abrupt reaction to seeing or hearing a distressing event involving someone close. Courts contrasted that experience with accumulated anxiety while watching a relative's condition deteriorate.

In Alcock v Chief Constable of South Yorkshire Police [1992] 1 AC 310, Lord Ackner described shock as sudden perception of a horrifying event that violently disturbed the mind. Later decisions treated this language as a separate legal restriction. That approach is important to understanding the older cases, but it is not a complete statement of the law after 2024. [1]

> **Current rule:** Paul v Royal Wolverhampton NHS Trust [2024] UKSC 1 rejected an additional requirement of a "sudden shock to the nervous system". It also rejected a separate test requiring an event to be sufficiently "horrifying". Do not present either as an extra current condition. [1, paras 71-78]

2. What remains necessary?

A claim for psychiatric illness still requires a medically recognised illness and a causal connection to the defendant's breach. Ordinary sorrow, fear or distress alone is insufficient. A close relative's suffering does not establish a diagnosis automatically; the claimant's own illness must be proved.

Within the established secondary-victim accident category, the claimant must satisfy the applicable duty and foreseeability requirements and the **Alcock controls**: close ties of love and affection; closeness in time and space to the accident or its immediate aftermath; and direct perception rather than merely receiving news. [1]

3. Three questions that must not be confused

Medical: What illness does the claimant have? **Causal:** What experience caused it? **Legal:** Does the defendant owe this claimant the relevant duty? A genuine illness can exist even where a claim fails. Equally, sympathy does not prove illness or liability.

This guide concerns secondary-victim psychiatric-injury claims. It does not apply every accident-based limit to all contexts, such as a worker's own claim arising from work-related stress.

The older medical cases

4. Sion v Hampstead Health Authority (1994)

Facts: A young man was injured in a motorcycle accident and treated in hospital. His father remained at his bedside for fourteen days, watching him deteriorate, fall into a coma and die. The father alleged that the hospital negligently failed to diagnose continuing internal bleeding and that this caused the death and his own psychiatric illness.

Who was sued? The hospital authority, not the person responsible for the road accident. This distinction matters: the claim concerned the father's experience of the hospital treatment and decline. It should not be described as a claim based on his witnessing the original collision.

Outcome: The claim was struck out, and the Court of Appeal upheld that result. Applying the then-used shock requirement, it held that the alleged illness arose from a gradual process, not a sudden shocking event; by the time of death, the death was expected. The alleged negligence was assumed for this preliminary legal assessment, not established after a full trial. [1, paras 62-63]

5. Liverpool Women's Hospital NHS Foundation Trust v Ronayne

Case: [2015] EWCA Civ 588. Negligence during Mrs Ronayne's surgery led to serious illness and emergency treatment. Her husband saw her connected to medical equipment before further surgery and later unconscious, ventilated and badly swollen. She spent a lengthy period in intensive care but eventually recovered.

Diagnosis: Mr Ronayne alleged PTSD. The trial judge rejected that diagnosis but found psychiatric illness, treated in the appeal as an adjustment disorder. Do not say that the court established PTSD merely because the source transcript uses that label.

Outcome: The husband initially won at trial, but the hospital's appeal succeeded and his claim failed. The Court of Appeal applied the older sudden-shock and horrifying-event approach. It treated what he experienced as a sequence of distressing events rather than the required sudden experience and doubted the causal analysis. [2]

How to use these cases now: Explain what the courts decided at the time, then address Paul. The shock-based reasoning is not a current freestanding test, and removing it does not automatically reverse the outcomes or establish every remaining element.

The position after Paul

6. Paul: causation without an extra shock mechanism

On 11 January 2024, the Supreme Court clarified that the remarks about sudden shock in Alcock did not establish an additional restriction on secondary-victim recovery. A claimant need not prove a particular neurological or psychological mechanism by which witnessing the accident caused the illness. The causal connection itself still has to be proved. [1, paras 73-74]

The Court also rejected a separate threshold of how “horrifying” an event must be. That is different from asking whether psychiatric illness was reasonably foreseeable in the claimant’s position. Removing an extra horror test does not remove foreseeability. [1, paras 75-78]

7. A slow diagnosis is not the same as a slow accumulation

Invented example A: A mother witnesses a negligently caused collision seriously injuring her child. Symptoms emerge afterwards, and she receives a diagnosis months later. The delay in diagnosis does not, by itself, defeat her claim. She must establish recognised illness, causation and the other applicable requirements.

Invented example B: A relative becomes ill while repeatedly visiting a patient whose disease deteriorates over weeks. Removing the shock requirement does not make that claim automatically recoverable. The relevant duty, the accident-based limits and the other requirements must still be examined.

> **Keep distinct:** the time of the accident; when the claimant perceived it or its immediate aftermath; the process by which illness developed; and the date of diagnosis. A diagnosis need not be made at the scene of an accident.

8. Why did the families in Paul still lose?

The Supreme Court dismissed the appeals in Paul, Polmear and Purchase. The majority declined to extend the established accident-based exception to witnessing the relevant medical crises or deaths from disease caused by earlier diagnostic or treatment negligence. The duty owed to the patient did not extend in those circumstances to shielding family members from the experience. [1]

This concerns the scope of the duty, not whether the families had suffered genuine illnesses. Nor is it a universal rule that medical negligence can never involve an accident. An account of Sion or Ronayne must therefore distinguish their historical shock reasoning from the modern duty inquiry.

Exam method: If an older authority uses sudden shock, identify the old reasoning, cite Paul’s correction, then test the recognised illness, causation, foreseeability, relevant accident and Alcock controls.

Check your understanding

1. Is sudden shock an additional current requirement for a secondary-victim claim?
Answer: No. Paul rejected a separate “sudden shock to the nervous system” requirement.

2. Did Paul remove the need for a medically recognised illness or causation?
Answer: No. Both remain necessary in a claim for psychiatric illness.

3. Who was the defendant in Sion?
Answer: The hospital authority. The alleged wrong was negligent treatment after the son’s motorcycle accident, not negligent driving by that defendant.

4. What happened in Ronayne?
Answer: The husband won at trial but lost when the hospital appealed. The trial judge rejected PTSD; the psychiatric illness considered on appeal was an adjustment disorder.

5. Must the claimant now prove that the event was separately “horrifying” enough?
Answer: No. Paul rejected that extra test. Reasonable foreseeability of psychiatric illness remains relevant.

6. Does a diagnosis made months later necessarily defeat an accident claim?
Answer: No. Timing of diagnosis and the causal connection to the accident are different questions. All remaining requirements still matter.

7. Did rejecting sudden shock mean that the Paul families won?
Answer: No. Their appeals failed on the scope of the duty and the accident-based exception.

The central lesson

Historically: courts often demanded a sudden shocking or horrifying event. **After Paul:** those extra tests are rejected, but recognised illness, causation, foreseeability and the established limits on secondary-victim duties remain.

Sources and further reading

[1] *Paul v Royal Wolverhampton NHS Trust; Polmear v Royal Cornwall Hospitals NHS Trust; Purchase v Ahmed* [2024] UKSC 1: paras 62-63 (Sion), 69-78 (Ronayne and the rejected tests), 138-143 (medical duty and result). It also discusses *Alcock* [1992] 1 AC 310. Supreme Court judgment.

[2] *Liverpool Women’s Hospital NHS Foundation Trust v Ronayne* [2015] EWCA Civ 588, especially paras 1-9 and 38-48. Court of Appeal judgment, The National Archives. *Sion v Hampstead Health Authority* [1994] 5 Med LR 170 is discussed in [1].